



Care Indeed Home Health

Mock Patient Evaluation Document · Version 1.0 MOCK · Training Date
July 1, 2026

MOCK TRAINING RECORD
SYNTHETIC UAT DATA
NOT FOR CLINICAL USE
DO NOT UPLOAD TO LIVE PATIENT
CHART

EVALUATION 4 — SEVERE ACUITY

Primary Diagnosis: Post-Liver Transplant with Immunosuppression, Surgical Wound Complications, New-Onset Diabetes, and Psychiatric Co-Morbidity

PATIENT DEMOGRAPHICS

Field	Detail
Patient Name	Andrei Vasile Constantinescu
Date of Birth	04/22/1965
Age	60
Sex	Male
Race/Ethnicity	White / Romanian American
Primary Language	English (fluent); Romanian (native — elderly parents in home speak limited English)
Interpreter Needed	Yes — Romanian interpreter needed when communicating with parents (they are secondary caregivers)
Marital Status	Divorced (2019)
Medical Record Number	CI-2025-00439
Address of Service	4715 Eucalyptus Drive, Apt 2C (second floor, elevator building), Menlo Park, CA 94025
Cell Phone	(650) 555-0648
Email	andrei.const65@email.com
County	San Mateo

EMERGENCY CONTACTS

Priority	Name	Relationship	Phone	Lives with Patient?
1	Elena Constantinescu	Mother (age 83, limited English, mild cognitive impairment per family report)	(650) 555-0649	Yes — moved in post-transplant
2	Mihai Constantinescu	Father (age 86, limited English, mobility impaired — uses walker, hard of hearing)	(650) 555-0649 (same line)	Yes — moved in post-transplant
3	Irina Petrescu	Sister	(408) 555-0912	No — San Jose (visits weekends)
4	Stanford Transplant Coordinator	Team	(650) 555-1600 (24-hour transplant line)	N/A

LIVING SITUATION

Field	Detail
Lives With	Elderly parents (mother 83, father 86) — neither is capable of being a reliable independent caregiver
Residence Type	2-bedroom apartment (2nd floor), elevator access, rented
Stairs	None (elevator)
Pets	None (immunosuppression — advised against new pets)
DME in Home	Hospital bed, bedside commode, rolling walker, wound care supplies (provided by transplant center), home IV pole, digital scale, thermometer, BP monitor, glucometer
Special Considerations	Immunocompromised patient — strict infection control required. No fresh flowers, no standing water, no construction/renovation nearby, visitors must be screened for illness.

PAYER INFORMATION

Priority	Payer	ID / Policy Number
Primary	Medicare Part A (ESRD/Transplant)	2NM8-GK4-YP62
Secondary	Medicare Part B	Same
Tertiary	Medi-Cal (SLMB — pays Medicare premiums + limited benefits)	94052817C

PHYSICIAN INFORMATION

Role	Name	Specialty	Phone
Attending / Certifying	Dr. Michelle Park	Transplant Hepatology	(650) 555-1605
Transplant Surgeon	Dr. Richard Abramson	Transplant Surgery	(650) 555-1610
PCP	Dr. Omar Farooq	Internal Medicine	(650) 555-0330
Psychiatrist	Dr. Lisa Cho	Psychiatry	(650) 555-1780
Endocrinologist	Dr. Anita Desai	Endocrinology	(650) 555-1900
Infectious Disease	Dr. Christopher Wynn	Infectious Disease	(650) 555-1655

F2F Encounter Date: 06/08/2025 (inpatient, post-transplant day 19) F2F Performed By: Dr. Michelle Park

REFERRAL / HOSPITALIZATION HISTORY

Event	Facility	Dates	Reason
Liver Transplant	Stanford Medical Center	05/20/2025 – 06/10/2025	Orthotopic liver transplant (deceased donor) for ESLD secondary to alcohol-related cirrhosis (MELD 34); complicated by: bile leak (re-operation 05/25), surgical site infection (MRSA — treated), acute rejection episode (treated with pulse steroids), new-onset diabetes after transplant (NODAT)
Inpatient Rehabilitation	Stanford Transitional Care	06/10/2025 – 06/18/2025	Post-transplant reconditioning

Pre-Transplant History:

Alcohol-related cirrhosis diagnosed 2020; listed for transplant 2023
Required to complete substance abuse treatment and maintain sobriety x 6 months pre-transplant
Last alcohol use: 11/2023 (per patient report; confirmed by transplant team monitoring)
History of alcohol use disorder, severe (in sustained remission per DSM-5)
History of major depressive disorder, recurrent, severe with prior suicidal ideation (2019 — divorce-related; no attempts)
History of generalized anxiety disorder

Start of Care Date: 06/19/2025

DIAGNOSES

Priority	ICD-10	Diagnosis
Primary	Z94.4	Liver transplant status (orthotopic, deceased donor, 05/20/2025)
Surgical	T86.41	Liver transplant rejection (acute cellular — treated, resolving)
Surgical	T81.41XA	Infection following a procedure, surgical site, organ and space (MRSA — treated, completing IV antibiotics)
Surgical	K83.19	Other obstruction of bile duct (bile leak — repaired 05/25/2025, resolved)
Secondary	E13.9	Other specified diabetes mellitus without complications (NODAT — new-onset diabetes after transplant, steroid/tacrolimus-induced)
Secondary	Z79.4	Long-term (current) use of insulin
Secondary	Z79.899	Other long-term (current) drug therapy — immunosuppressants
Secondary	B95.62	MRSA as the cause of diseases classified elsewhere (surgical site — completing treatment)
Secondary	F10.21	Alcohol use disorder, in sustained remission
Secondary	F33.2	Major depressive disorder, recurrent, severe without psychotic features (current episode — pre-existing, worsened post-transplant)
Secondary	F41.1	Generalized anxiety disorder
Secondary	K74.60	Unspecified cirrhosis of liver (native liver — removed)
Secondary	D64.9	Anemia, unspecified (post-surgical, Hgb 8.9)
Secondary	I10	Essential hypertension (new — tacrolimus-related)
Secondary	N18.4	Chronic kidney disease, stage 4 (GFR 22 — calcineurin inhibitor nephrotoxicity + pre-existing hepatorenal component)
Secondary	E87.1	Hypo-osmolality and hyponatremia (Na 131 at discharge — chronic, monitoring)
Secondary	D69.6	Thrombocytopenia, unspecified (platelets 78K at discharge)

ALLERGIES

Allergen	Reaction	Severity
Sulfa drugs (Bactrim/TMP-SMX)	Stevens-Johnson Syndrome (2003)	LIFE-THREATENING
Vancomycin	Red Man Syndrome (rate-related; tolerated with slow infusion + premedication)	Moderate — not a true allergy; requires slow infusion protocol
Shellfish	Angioedema	Moderate
Latex	Contact dermatitis	Mild

CURRENT MEDICATIONS (Transplant patients have complex, time-sensitive, interaction-critical regimens)

#	Medication	Dose	Route	Frequency	Purpose	Critical Notes
1	Tacrolimus (Prograf)	3 mg	PO	BID (0800, 2000 — exact 12-hour intervals)	Primary immunosuppression	HIGH ALERT. Trough levels drawn q Monday AM (target 8-12 ng/mL). Take on empty stomach (1 hr before or 2 hrs after food). NO grapefruit. Drug interactions critical — consult transplant pharmacist before any new medication.
2	Mycophenolate mofetil (CellCept)	1000 mg	PO	BID (0900, 2100)	Immunosuppression (adjunct)	Take on empty stomach. Do not crush or open capsules. GI side effects common.
3	Prednisone	15 mg	PO	Daily AM (tapering: reduce by 2.5 mg every 2 weeks per transplant team)	Immunosuppression (steroid taper post-rejection)	Monitor glucose closely (steroid-induced diabetes); adrenal suppression risk — do not abruptly discontinue; bone health concern.
4	Daptomycin (Cubicin)	500 mg	IV	Daily x 14 more days (through 07/03/2025)	MRSA surgical site infection (completing course)	IV antibiotic — requires PICC line care. Infuse over 30 min. Monitor CPK weekly (rhabdomyolysis risk). Incompatible with dextrose-containing solutions (use NS only).
5	Insulin glargine (Lantus)	18 units	SubQ	Daily HS (bedtime)	NODAT — basal insulin	Dose adjustment per sliding scale and endocrinology guidance.
6	Insulin lispro (Humalog)	Per sliding scale	SubQ	Before meals (AC)	NODAT — prandial insulin	Sliding scale provided by endocrinology. FSBS: QAC + HS (4x/day).
7	Valganciclovir (Valcyte)	450 mg	PO	Daily	CMV prophylaxis (donor CMV+/recipient CMV-: high risk)	Monitor CBC (neutropenia risk). Renal dose adjusted for GFR 22.
8	Atovaquone (Mepron)	1500 mg	PO	Daily with fatty meal	PJP (Pneumocystis) prophylaxis (cannot use TMP-SMX — SJS allergy)	Suspension — shake well. Must take with food (fat enhances absorption).
9	Fluconazole (Diflucan)	100 mg	PO	Daily	Fungal prophylaxis	Drug interaction with tacrolimus (increases tacrolimus levels) — levels being monitored closely.
10	Amlodipine	10 mg	PO	Daily	HTN (tacrolimus-induced)	Drug interaction with tacrolimus — dose stabilized.

11	Pantoprazole (Protonix)	40 mg	PO	Daily (AM, 30 min before food)	GI prophylaxis (steroids + immunosuppression)	
12	Sertraline (Zoloft)	150 mg	PO	Daily AM	Depression / anxiety	Drug interaction note: monitor with tacrolimus; serotonin levels.
13	Lorazepam (Ativan)	0.5 mg	PO	PRN (max BID) for acute anxiety	Anxiety — breakthrough	Controlled substance. Monitor for overuse; assess for relapse risk in context of AUD history. Prescriber aware of AUD history.
14	Epoetin alfa (Procrit)	10,000 units	SubQ	Weekly (Monday)	Anemia (CKD + post-surgical)	Monitor Hgb weekly; target 10-11 g/dL.
15	Calcium carbonate + Vit D3	600 mg / 400 IU	PO	BID	Bone health (steroid-induced osteoporosis prevention)	Take 2 hrs apart from tacrolimus (calcium can affect absorption).
16	Ferrous sulfate	325 mg	PO	Daily	Iron deficiency anemia	Take 2 hrs apart from tacrolimus and pantoprazole.
17	Ursodiol (Actigall)	300 mg	PO	TID	Bile duct protection / liver support	
18	Magnesium oxide	400 mg	PO	BID	Hypomagnesemia (tacrolimus-induced)	Monitor Mg levels.
19	Sodium bicarbonate	650 mg	PO	TID	Metabolic acidosis (CKD)	Monitor bicarb levels.

TOTAL MEDICATIONS: 19 Controlled Substances: Lorazepam 0.5 mg (monitored — AUD history) Sharps in Home: Yes — insulin syringes, Procrit syringes PICC Line: Yes — right basilic vein, 4 Fr single-lumen, inserted 06/01/2025, confirmed by CXR tip at cavo-atrial junction. Dressing change: every 7 days and PRN (CHG-impregnated dressing). Flush: NS 10 mL before and after medication; heparin lock per protocol when not in use.

CLINICAL ASSESSMENT — VITAL SIGNS AT ADMISSION

Parameter	Value	Abnormal?
BP	152/94	Elevated (tacrolimus-related HTN)
HR	92, regular	Mildly elevated (anemia, deconditioning)
RR	18	WNL
Temp	99.3°F	Borderline — monitor closely in immunosuppressed patient (threshold for concern is lower: contact transplant team if $\geq 100.0^{\circ}\text{F}$)
SpO2	96% RA	WNL
Weight	168 lbs (76.2 kg) — pre-transplant weight 185 lbs	Loss of 17 lbs — expected post-transplant
BMI	24.8	WNL
Pain	6/10 (surgical site — right upper quadrant), 3/10 generalized body aches	Yes
Blood Glucose	267 mg/dL (fasting)	Significantly elevated — NODAT + steroids

WOUND ASSESSMENT

Primary Surgical Wound — Bilateral Subcostal (Chevron) Incision:

Parameter	Finding
Location	Bilateral subcostal / chevron incision with midline extension to xiphoid
Total Length	Approximately 38 cm
Closure	Subcuticular absorbable sutures + Dermabond (surgical glue) — majority well-approximated
Complication Area	4 cm segment at right lateral incision line: previously debrided (MRSA infection, 06/02/2025); currently granulating
Wound Bed (complication area)	70% granulation, 30% slough (yellow, adherent, thin)
Dimensions (complication area)	4.0 cm L x 1.2 cm W x 0.8 cm D
Drainage	Scant serosanguineous from wound; no purulence since daptomycin initiated
Peri wound	Mild induration; erythema resolved (was 3 cm; now <0.5 cm)
MRSA status	Most recent wound culture (06/14/2025): no growth. Pending final clearance culture per ID team.
Current Wound Care Orders	Cleanse with NS; light debridement of loose slough with gauze (no sharp debridement — thrombocytopenic); apply Aquacel Ag Advantage; ABD pad secondary; Mefix tape. Change every 48 hours and PRN.
Remainder of incision	Well-approximated, dry, healing. Steri-strips/Dermabond in place — leave intact until they fall off naturally.

Drain:

JP drain removed 06/08/2025 (inpatient).

No active drains.

PICC Site:

Parameter	Finding
Location	Right upper arm, basilic vein
Insertion Date	06/01/2025
Dressing	CHG-impregnated transparent dressing (Biopatch + Tegaderm)
Site Assessment	Clean, dry, intact. No erythema, swelling, drainage, or tenderness at insertion site.
Line Patency	Flushes without resistance; blood return present
Dressing Due	Change every 7 days (next due: 06/22/2025)
PICC Complications to Monitor	CLABSI (central line-associated bloodstream infection), phlebitis, thrombosis, air embolism, line migration/dislodgement, catheter fracture

HOMEBOUND STATUS

Criterion	Finding
Criterion 1	MET. Severe deconditioning post-transplant + prolonged hospitalization (29 days). Ambulates with rolling walker and standby assist, maximum distance 30 feet with fatigue. Surgical pain limits mobility. Additionally, immunosuppressed status makes community outings medically contraindicated — transplant team advises avoiding public spaces, crowds, and contact with potentially infectious individuals during early post-transplant period. Leaving home is a considerable and taxing effort.
Criterion 2	MET. Only planned outings are transplant clinic visits (weekly initially).
Homebound Status	CONFIRMED

ADVANCE DIRECTIVES

Document	Status
AHCD	Yes — executed 05/2025 (pre-transplant). Sister Irina Petrescu designated as agent (not parents due to parents' age/cognition/language).
POLST	Yes — Full treatment.
DNR	No — Full Code

PSYCHIATRIC / SUBSTANCE ABUSE CONSIDERATIONS

This patient has a documented history of:

Alcohol Use Disorder, severe, in sustained remission — last drink 11/2023. Transplant team has a zero-tolerance relapse policy. Patient is monitored by transplant social worker and attends AA meetings (currently via Zoom due to homebound status and immunosuppression). Any evidence of relapse must be reported to the transplant team immediately (may affect transplant follow-up and future candidacy if re-transplant is ever needed).

Major Depressive Disorder, recurrent, severe — pre-existing. Worsened post-transplant (post-surgical pain, deconditioning, financial stress from divorce/job loss, dependence on elderly parents). PHQ-9: 18 (moderately severe). Denies current suicidal ideation — "I just got a new liver, I'm not going to waste it" — but has history of SI (2019). Monitor closely.

Generalized Anxiety Disorder — significant health anxiety post-transplant ("Am I rejecting? Is this infection back?"). GAD-7: 15 (severe). Lorazepam prescribed PRN — use is being monitored given AUD history. Prescribing psychiatrist (Dr. Cho) is aware and has limited to 0.5 mg BID max with tracking.

Controlled substance monitoring: Lorazepam prescribed in limited quantity (qty 30/month). Wife no longer present for support. Parents unable to effectively monitor. Agency to track utilization and behavioral indicators of potential misuse and coordinate with psychiatrist.

SERVICES ORDERED

Service	Discipline	Frequency	Duration
Skilled Nursing	RN	Daily x 2 weeks (IV antibiotic + wound care + post-transplant monitoring), then 3x/week x 6 weeks	8 weeks minimum
Physical Therapy	PT	3x/week x 8 weeks	8 weeks
Occupational Therapy	OT	2x/week x 6 weeks	6 weeks
Medical Social Work	MSW	1x/week x 8 weeks	8 weeks
Home Health Aide	HHA	Daily (personal care, meal prep assist, light housekeeping — essential for immunosuppressed patient's environment)	Duration of need

SKILLED NURSING FOCUS AREAS

Immunosuppressant Management — Tacrolimus level monitoring (trough draws every Monday — fasted, drawn BEFORE morning dose); assess for rejection symptoms (fever, RUQ pain, jaundice, clay-colored stools, dark urine, elevated LFTs, fatigue); assess for toxicity (tremor, headache, GI symptoms, renal function changes). Critical: drug interaction management — NO new medications (Rx, OTC, herbal, supplements) without transplant team approval.

IV Antibiotic Administration — Daptomycin 500 mg IV daily: PICC access, NS flush, infuse over 30 min, flush post-infusion. Monitor for adverse effects (myopathy — CPK weekly; GI symptoms; eosinophilic pneumonia — rare). Assess PICC site per protocol. PICC dressing change weekly (CHG-impregnated). CLABSI bundle adherence.

Wound Care — Chevron incision complication area: wound care per orders every 48 hours. Measure, photograph, document. Assess for MRSA recurrence. Coordinate with transplant surgery and ID.

Diabetes Management (NODAT) — FSBS QAC + HS (4x/day). Insulin glargine and lispro administration/teaching. Sliding scale management. Coordinate with endocrinology for dose adjustments. Steroid taper will significantly affect glucose — expect glucose to decrease as prednisone decreases; risk of hypoglycemia during taper. Teach hypo/hyperglycemia recognition.

Lab Coordination — Weekly: tacrolimus trough, CMP, CBC with diff, Mg, Phos, LFTs, CPK, INR. CMV PCR per transplant schedule. Coordinate all draws with transplant team.

Infection Surveillance — This patient is profoundly immunosuppressed. Temperature check twice daily by patient/parents (contact transplant team if $\geq 100.0^{\circ}\text{F}$ — NOT 100.4°F as in non-immunosuppressed patients; threshold is lower). Assess for opportunistic infections: CMV (fever, GI symptoms, vision changes), PJP (dry cough, dyspnea), fungal (oral thrush, skin lesions), herpes simplex/zoster. Mask and gloves for ALL home visits. Environmental infection control education.

Renal Monitoring — CKD Stage 4 with tacrolimus nephrotoxicity risk. Strict I&O. Daily weight. BMP weekly. Educate on fluid management. Monitor for uremia symptoms.

Psychiatric Monitoring — PHQ-9 every 2 weeks. GAD-7 monthly. Assess medication compliance (sertraline, lorazepam — track lorazepam use). Screen for substance relapse indicators. Coordinate with Dr. Cho (psychiatry) and transplant social worker.

Caregiver Assessment — Parents are unreliable caregivers (age, cognition, language). Sister visits weekends only. Identify and address caregiver gaps. Explore paid caregiver options, adult day health, meal delivery, medication management technology (automated pill dispenser with alarms).

PLAN OF CARE GOALS (60-DAY CERTIFICATION PERIOD)

#	Goal	Target
1	No transplant rejection episodes (confirmed by LFTs and tacrolimus levels WNL)	08/17/2025
2	MRSA infection resolved; daptomycin completed; PICC removed	07/10/2025
3	Surgical wound fully epithelialized	08/17/2025
4	FSBS within target range (100-200 fasting, <250 post-prandial) ≥70% of readings	08/17/2025
5	Patient demonstrates independent insulin administration and FSBS monitoring	07/15/2025
6	Patient verbalizes understanding of all 19 medications (purpose, dose, timing, key interactions, side effects)	07/31/2025
7	Tacrolimus levels within target range (8-12) for ≥80% of draws	08/17/2025
8	No opportunistic infections	08/17/2025
9	Ambulate 200 feet with least restrictive device and standby assist	08/17/2025
10	Independent with ADLs (bathing, dressing, grooming, toileting)	08/01/2025
11	PHQ-9 decreased to ≤14 (from 18)	08/17/2025
12	Zero substance relapse	08/17/2025
13	Sustainable caregiver support plan in place	07/31/2025
14	Zero falls, zero hospital readmissions	08/17/2025
15	Renal function stable (GFR >20)	08/17/2025

PATIENT SIGNATURES — EVALUATION 4

Patient Signature: _____ Date: _____

Print Name: Andrei Vasile Constantinescu

Clinician Signature: _____ Date: _____

Patient / Representative / Conservator

Mock signature area — training only.

Evaluating Clinician

Mock signature area — training only.

Caregiver / Interpreter if applicable

Mock signature area — training only.

Date / Attestation

Mock signature area — training only.

